

MP-103: Continuous Glucose Monitors (CGM)

Bellcourt Medical Policy | Version 1 | Status: RETIRED (superseded 2025-12-31) | Effective for dates of service 2024-01-01 through 2025-12-31

Field	Value
Policy number	MP-103
Service code(s)	BHA-DME-2103
Applies to	All clients administered by Bellcourt unless the client's plan document or delegation addendum states otherwise
Owner	Clinical Policy Committee
Version	v1

1. Purpose and scope

This policy sets out the clinical criteria Bellcourt reviewers use to decide whether the service described in this policy (Continuous Glucose Monitors (CGM)) is medically necessary. It is applied to prior authorization requests where the date of service falls within the effective period stated above. A client's plan document may exclude this service or impose additional requirements; plan provisions are checked first.

2. Definitions

Medically necessary: services that are appropriate and consistent with the diagnosis, in accordance with generally accepted standards of medical practice, and not primarily for the convenience of the member or provider.

3. Coverage criteria

The service is considered medically necessary when the criteria below are met, read together using the AND / OR connectors shown:

- 1 Diagnosis of diabetes mellitus (type 1 or type 2); AND
- 2 Intensive insulin therapy: 3 or more daily insulin injections, or use of an insulin pump.

4. Not medically necessary / limitations

- Diabetes managed with oral agents or basal insulin alone.
- Use for non-diabetic indications.

5. Documentation requirements

Diagnosis, current medication list showing insulin regimen, hypoglycemia history if relevant.

If required documentation is missing, the request must be pended and the provider contacted for information. It must not be denied for lack of documentation until the information request has gone unanswered within the regulatory timeframe.

6. Version history

Version	Effective	Summary
v1	2024-01-01 to 2025-12-31	Initial version.
v2	2026-01-01	Expanded to any insulin use or documented problematic hypoglycemia.